QUESTIONS BANK 3

1.Which of the following statements about breastfeeding is CORRECT?
A/Breastfed infants have fewer allergies
B/Women who breastfeed have a lower risk of breast cancer in later life
C/Breastfed infants gain more weight and gain weight more quickly
D/A+B✅
E/ A+B+C

2.Which of the following statements about postpartum hemorrhage (PPH) is TRUE?
A/PPH should be diagnosed if there is z500 mL estimated blood loss after vaginal delivery
B/ PPH should be diagnosed if there is a drop in hematocrit of 210% after delivery
C/PPH complicates approximately 5% of all deliveries
D/ A+C
E/ A+B+C✅

3.Episiotomy is curently performed in approximately what percentage of vaginal deliveries in the USA?
A />10% ✅
B/ 15%
C/20%
D/ 25%
E/ 33%

4.Which of the following statements regarding cesarean section delivery is TRUE?
A/Maternal morbidity is significantly increased with cesarean compared with vaginal delivery✅
B/Elective cesarean section delivery can be performed before 39 weeks gestation without documenting fetal lung maturity

C/Electives surgeries (such as I myomectomy) c can be performed safely at the time of cesarean section
D/Adesire for permanent sterilization by bilateral tubal ligation is an acceptable indication for cesarean section delivery
E/One-third of all cesarean section deliveries in the USA are elective cesarean sections performed on maternal request

5.Non-reassuring elements of intrapartum fetal heart rate monitoring patterns include which of the following?
A/Minimal or absent variability
B/ Absence of accelerations
C/ Bradycardia or tachycardia
D/ A+B
E/ B+C
F/ A+C✅

6. 37-year-old G2P 1presents at 40 weeks for elective induction of labor. Her prior delivery was by cesarean section for breech presentation at term. Which of the following methods of inducing labor is absolutely contraindicated in this setting?
A/Oxytocin
B/Membrane stripping
C/Transcervical Foley catheter
D/ Vaginal administration of prostaglandin E2✅
E/Amniotomy

7.The second stage of labor is defined as which of the following?
A/The interval from onset of contractions to the beginning of cervical dilation
B/The time from full cervical dilation to delivery of the fetus✅
C/The duration of maternal pushing efforts
D/The time from delivery of the fetus until delivery of the placenta
E/The time from delivery of the placenta to discharge from the hospital

8. 28-year-old G4P2+1at 31 weeks' gestation presents to your office complaining of leakage of clear fluid from the vagina. She denies abdominal pain or vaginal bleeding. On sterile speculum exam, you confirm the presence of amniotic fluid and the cervix appears closed.

Which of the following represents the most appropriate management option?
A/Discharge home with a follow-up visit in clinic in 24-48 hours
B/Discharge home only if the ultrasound examination shows a normal amniotic fluid
yolume
C/Admission for augmentation of labor
D/ Admission for antibiotics and antenatal corticosteroids✅
E/Admission for cesarean section for breech presentation

9. A 39-year-old G2P1 presents at 18weeks' gestation for her scheduled fetal anatomic survey. During the examination, the fetal anatomy is noted to be normal, but
the cervical length is measured at 15 mm. Despite counseling about the lack of consistent data showing a benefit in this setting, the patient requests that a cervical cerclage be placed. Which of the following, if present, would be considered an absolute contraindication to cervical cerclage placement?
A/Advanced maternal age
B/Prolapse of fetal membranes into the vagina
C/ Positive fetal fibronectin test
D/Rupture of fetal membranes✅
E/ Placenta previa

10. 24-year-old G2P1 presents at 26 weeks' gestation with regular, painful uterine contractions and cervical dilation to 4 cm. You perform an amniocentesis to exclude intra-amniotic infection. Which of the following risk factors is most strongly associated with her diagnosis?
A/negative fetal fibronectin test between 22 and 24 weeks
B/ prior term birth
C/Gram-positive cocci in amniotic fluid✅
D/Cigarette smoking
E/Chlamydia infection at initial prenatal visit

11.What is the single greatest cause of intrauterine growth restriction (IUGR worldwide?
A/Maternal cocaine use
B/Malaria✅

C/Uteroplacental insufficiency
Q/ Fetal chromosomal abnormality
E /Fetal alcohol syndrome

12. In monozygous twin pregnancy, what type of chorionicity results when the blastocyst divides between days 9 and 12 post-conception?
A/Conjoined twins
B/Dichorionic/diamniotic placentation
C/Monochorionic/monoamniotic placentation✅
D/Monochorionic/diamniotic placentation

13.What is the most common cause of non-immune fetal hydrops in the USA?
A/ Maternal parvovirus B19 infection
B/Fetal chromosomal abnormality
C/ Fetal anemia
D/ Fetal cardiac anomaly
E/ ldiopathic✅

14.At what gestational age is the amniotic fluid volume at its maximum?
A/ 22 weeks
B/ 26 weeks
C/ 30 weeks✅
D/ 34 weeks
E/ 38 weeks

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B/Fetal chromosomal abnormality
C/ Fetal anemia
D/ Fetal cardiac anomaly
E/ ldiopathic✅

16.What is the single most useful test in identifying the cause of an as yet unexplained term intrauterine fetal demise?
A/Serologic testing for antiphospholipid antibody syndrome

B/ Kleihauer-Betke test
C/ Fetal autopsy✅
D/ Urine toxicology
K/ Serologic testing for TORCH infections

17.Which of the folowing medications is NoT associated with central nervous system (cNS) abnormalities?
A/Valproic acid
B/Methotrexate
C/ Tegretol
D/ Vitamin A
E/Thalidomide✅

18.Medications that may be acceptable for use in pregnancy include which of the following FDA categories?
A/Category A
B/Category B
C/Category G
D/CategoryD
E/All of the above✅

19.Which of the following drugs is a known fetal teratogen?
A/Marijuana
B/ Alcohol✅
C/ Heroin
D/Caffeine
E/ Nicotine

20.All of the following pregnancy neonatal complications are associated with maternal cigarette smoking except one. Which is the exception?
A/ Low birthweight
B/ Sudden infant death syndrome (S1D8)
C/ Pre-eclampsia✅
D/ Preterm labor
E/ Preterm premature rupture of membranes (preterm PRoM)

21.Which of the following agents is a known teratogen in humans?

A/Vitamin A✅
B/ Vancomycin
C/oral contraceptives
D/Ampicillin
E/Metronidazole

22.Compared with non-pregnant women, where in the abdomen is the appendix located in the third trimester of pregnancy
A/ Lower in the abdomen
B/ Closer to the bladde
C/ ls position is unchanged
D/ Higher in the abdomen✅
E/ Closer to the cervix

23.Treatment of immune thrombocytopenic purpura (ITP) during pregnancy may include which of the following modalities?
A/ Corticosteroids
B/Plasmapheresis
C/Platelet transfusion
D/ IVIG
E/All of the above✅

24.Which of the following obstetric emergencies is associated with a maternal mortality rate of -90%?
A/ Myocardial infarction
B/ Pulmonary embolism
C/Postpartum hemorrhage
D/Eclampsia
E/Amniotic fluid embolism✅

25.Which of the following medications is recommended most commonly for the treatment of maternal hyperthyroidism in pregnancy?
b /Carbimazole
B/ Propylthiouracil✅
C /Levothyroxine
D/ Radioactive iodine
E/ Corticosteroids

26.In hypothyroid patients well controlled l on thyroid hormone replacement, how often should thyroid function tests be performed during pregnancy?
A/ One a day
B/ Every 1-2 weeks
C/ Every 1-2 months✅
D/ Once trimester
E/They do not need to be checked

27.Poorly controlled maternal thyroid disease is associated with which of the following pregnancy complications?
A/Stillbirth
B/Intrauterine growth restriction
C/Preterm delivery
D/ Pre-eclampsia
E/All of the above✅

28.How do circulating TSH levels change during pregnancy?
A/ Increased
B/Decreased
C /Remain unchanged✅
D /Increase then decrease
E /Decrease then increase

29.Which of the following maternal cardiac lesions is associated with a pregnancy-associated maternal mortality rate of 50% or more?
A/Surgically corrected tetralogy of Fallot
B/ Ventricular septal defect (vSD)
C/ Marfan syndrome with an aortic root diameter of 5 cm✅
D/ Previous myocardial infarction
E/ Prosthetic heart valve

30.Which of the folowing patients should receive antibiotic chemoprophylaxis against bacterial endocarditis during delivery?
A/ Small asymptomatic atrial septal defect (AsD)
B/Prosthetic heart valve✅

C/Mitral valve prolapse
D/ Cardiomyopathy
E/Wolf-Parkinson-White syndrome

31.Which of the following tests is the most accurate in diagnosing a pulmonary embolus during pregnancy?
A/Ventilation-perfusion (V/Q) scan
B/Spiral chest CT scan
C/ EKG
D/ Pulmonary angiography✅
E/ Serum D-dimer measurements

32.Which of the following pregnancy complications are NOT related to the presence of pregestational diabetes?
A/ Intrauterine growth restriction (IUGR)
B /Atrial septal defects (ASDs)
C/ Gastroschisis✅
D/Pre-eclampsia
E/ Miscarriage

33.What is the primary goal of diagnosing and treating gestational diabetes?
A/To prevent congenital malformations
B/To prevent miscarriage
C/To decrease the incidence of fetal macrosomia✅
D/To reduce the risk of type 2 diabetes in the offspring
E/All of the above

34.When is magnesium seizure prophylaxis typically given to try to prevent an eclamptic convulsion?
A/Anteparum
B/ Intrapartum and the immediate postpartum period✅
C/ 2 weeks postpartum
D/weeks postpartum

35.Which of the following findings would confirm a diagnosis of 'severe' pre eclampsia?

A/ Serum creatinine of 0.8 mgldl
B/Serum AST (aspartate transaminase) 140 mg dl✅
C/1t proteinuria on dipstick
D/ p BP 158[96 mmHg
E/ uric acid of 5.8 mgldl

36.Magnesium sulfate is commonly administered to women with pre-eclampsia What is the primary goal of this therapy?
A/To lower the BP
B/To prevent preterm labor
C/ For neuroprotection of the fetus
D/ To prevent seizures✅
E/ To dilate the cerebral vasculature and prevent headache

37.A 39-year-old G2P1 presents at 38 weeks' gestation with a BP 174/112 mmHg and 2+ proteinuria. You proceed with induction of labor. Your primary aim in lowering her BP is to prevent which of the following complications?
A/Seizures (eclampsia)
B/Renal failure
C/Pulmonary edema
D/Preterm labor
E/Stroke✅

38.A healthy, previously normotensive, 26-year-old G1Po presents to your clinic at 34 weeks' gestation with a BP of 142/94 mmHg. What test should you check next?
A/ Type and screen
B/Complete blood count
C/ Liver function tests
D/Urine dip✅
E/Uric acid

39.Congenital syndromes related to viral infections are most likely to occur if the mother contracts the illness during which of the following trimesters?
A/ in the first trimester✅
B/ 9 In the second trimester

C/In the third trimester
D/ Postpartum

40.Which of the following infections are sexually transmitted?
A/ HIV
B/Herpes
C/syphilis
D/chlamydia infection
E/All of the above✅

41.patient presents for her frst prenatal visit at 10 weeks' gestation. Her routine prenatal intake laboratory tests reveal that she is not immune to rubella. what should you recommend?
A/Administer MMR vaccination now
B/Administer MMR vaccination at 28 weeks
C/ Administer MMR vaccination postpartum✅
D/ She does not need MMR vaccination

42.A patient who is a chronic hepatitis B carrier delivers vaginally at 39 weeks gestation. The baby should receive which of the following therapies on the first day of life?
A/Hepatitis B immune globulin (HBIG)
B/Hepatitis B vaccination
C/HBIG and hepatitis B vaccination✅
D/ None of the above

43.A 24-year-old G3P2 presents to your office for the first time at 26 weeks gestation. She recently arrived from West Africa and has had no prenatal care. You plant a PPD (purified protein derivative), which at 72 hours measures 12 mm induration. Which of the following management options is most appropriate?
A/ Do not do any further testing because she is asymptomatic
B/ Defer further evaluation until after the pregnancy
C/Initiate INH (isoniazid) therapy immediately
D/ Initiate combined INH and ethambutol therapy immediately
E/ Obtain a chest radiograph✅

44.An 18-year-old primagravida presents at 30 weeks' gestation complaining of malodorous vaginal discharge. A wet prep reveals motile flagellated organisms. The treatment plan would include which of the following antibiotics?

A/Ampicllin
B/ Metronidezole✅
C/ Diflucan
D/ Gentamicin
E/Bactrim

45.A 24-year-old G2P 1presents at 36 weeks' gestation in active labor. Her GBS culture is not available. Which of the following represents the most appropriate management in this setting?

A/ Do nothing
B/Administer GBS chemoprophylaxis empirically✅
C/Administer antibiotics only if she develops a fever
D/ Send a GBS perineal culture and wait for the result to decide whether or not to give antibiotics
E/Review her GBS status from her prior pregnancy to decide whether or not to give antibiotics

46.Which of the following organisms is the most common cause of early onset neonatal conjunctivitis?

A/GBS
B/Toxoplasmosis
C/ Chlamydia✅
D/ Trichomoniasis
E/ E. coli

47. Chorioamnionitis (intra-amniotic infection) is typically caused by which of the following organisms?

A/ group B p-hemolytic streptococcus (6Bs)
B/Escherichia coli
C /staphylococcus aureus
D /Ureaplasma urealyticum
E /Multiple pathogens✅

48.A number of screening tests are performed in all pregnancies at designated gestational ages. Which of thebfollowing screening tests is correctly matched with the gestational age?
A/Glucose load test at the first prenatal visit
B/Chlamydia test at 22-24 weeks
C/ Fetal anatomic survey at 34-36 weeks
D/Group B ß-hemolytic streptococcus (GBS) carrier status at 35-37 weeks✅
E/Rubella serology at the 6-week postpartum visit

49.Which of the following observations would NOT be expected to be seen on an obstetric ultrasound at 7 weeks' gestation?
A/Chorionic sac
B /Yok sac
C/ Embryo with cardiac activity
D/Embryo demonstrating movement
E/Physiologic bowel herniation✅

50.maternal "quad" screen performed at 18 weeks' gestation returns with the following results: a-fetoprotein (AFP) 0.5 MoM (multiples of the median), estriol 0.6 MoM, human chorionic gonadotropin (hCG) 2.0 MoM, and inhibin 1.8 MoM. Which fetal disorder is most likely given this serum analyte profile?
A/ Open neural tube defect
B/ Trisomy 21✅
C/ Trisomy 18
D/ Trisomy 13
E/ Turner syndrome

51.Levels of hCG in the maternal circulation typically peak at what level and at what gestational age?
A/ 100,000 mlU/mL at 10 weeks✅
B/ 10,000 mlU/mL at 10 weeks
C/ 10,000 mlU/mL at 20 weeks
D/ 100,000 mlU/mL at 40 weeks
E/ 10,000 mlU/mL at 40 weeks

52.All of the following are changes that occur to the fetal circulation within minutes of birth EXCEPT for which?
A/ Constriction of the umbilical vessels
B/Constriction of the ductus venosus
C/Constriction of the ductus arteriosus
D/ Constriction of the hepatic portal sinus✅
E/Closure of the foramen ovale

53.19-year old G1Po had a D&c for a complete mole. Post-evacuation, her BhCG levels plateaued at 5,000 mlU/mL and she was diagnosed with gestational trophoblastic neoplasia (GTN). The work-up confirms an anatomical stage I tumor with a WHO score of 5. What is the MOST appropriate management option? Your feedback Become a reviewer
A /Endometrial sampling to exclude placental-site trophoblastic tumor (PSTT)
B/Hysterectomy
C/ Methotrexate✅
D/ Combination etoposide, methotrexate, dactinomycin, cyclophosphamide, vincristine
(EMA-CO) therapy
E/ Serum analysis to exclude phantom hcG before proceeding with treatment

54.Which of the following is NoT a characteristic of a partial hydatidiform molar pregnancy!
A/ Presence of a fetus
B/ 69,XXX karyotype
C/Sonographic Snowstorm"appearance
D/ Risk of developing metastatic disease of 3-4%
E/ C and D✅

55.What is the MOST cost-effective method of screening for ovarian cancer?
A/ Transvaginal sonography
B/ Serum CA-125 tests
C/ Serum HE4 tests
D/ Combination of A and B
E/ None of the above✅

56.31-year-old Go presents after having her left tube and ovary removed. The
pathology shows a borderline (low malignant potential) tumor. Imaging is normal. What is the MOST appropriate treatment option?
A/ Hysterectomy, right tube/ovary removal, omentectomy, lymphadenectomy
B/Hysterectomy, right tube/ovary removal, omentectomy,
C/ Hysterectomy, right tube/ovary removal
D/Exploratory laparotomy, washings, staging biopsies
E/Observation✅

57.Which of the following is NOT a risk factor for endometrial cancer?
A/ Smoking✅
B /Polycystic ovarian syndrome(PCOS)
C/ Tamoxifen use
D/Germline MLH1 gene mutation
E/Postmenopausal use of botanicals

58.A 28-year-old Go with steroid-dependent sarcoidosis presents with multifocal vulvar lesions. Biopsy confirms carcinoma in situ. What is the MOST appropriate treatment option?
A/Skinning vulvectomy
B/ Complete radical vulvectomy
C/Radiation therapy
D/Topical 5-fluorouracil
E/ Laser ablation✅

59.46-year-old, healthy G4P4 had a cervical biopsy confirming the presence of invasive squamous carcinoma. Examination reveals a discrete 2-cm lesion. What is the BEST treatment?
A/Simple hysterectomy with sentinel node biopsy
B/ Radical hysterectomy with lymphadenectomy✅
C/ Exenteration
D/ Radical pelvic radiation
E/ Radical pelvic radiation with sensitizing chemotherapy

60.A 17-year-old Go is brought to the office by her mother upon learning of multiple instances of unprotected intercourse. Her examination is notable

only for the presence of a vaginal discharge. Which of the following is NOT a part of your comprehensive assessment?
A/ Cervical cultures
B/Cervical smear screening✅
C/ Contraceptive management
D/ Sexually transmitted infection (STI) counseling
E/ Saline wet-mount

61.A 52-year-old woman presents for her annual exam and gives a history that she has not had a menstrual period in 2 years. Which of the following is LEAST likely to be associated with the menopause?
A/Osteoporosis
B/Weight loss✅
C/ Vasomotor instability
D/ Genital atrophy
E/ Mood disturbances

62.A 37-year-old woman with an eating disorder (chronic bulimia) presents for infertility evaluation. Your work-up identifies ovulatory dysfunction with low levels of estrogen and gonadotropins. What is the BEST method of ovulation induction?
A/Clomiphene citrate
B/Human menopausal gonadotropin (hMG)✅
C/ Bromocryptine mesylate
D/ Pulsatile gonadotropin-releasing hormone (GnRH) therapy
E/None of the above - should proceed with ovum donation

63. A 34-year-old woman presents with primary infertility and inability to get pregnant over the past 18 months. She and her husband are in good health. Which test would not be part of the basic work-up?
A/Menstrual calendar
B/Day 3 FSH test
C/Semen analysis
D/Diagnostic laparoscopy✅
E/Thyroid panel

64.What is the MOST proven cause of recurrent pregnancy loss (RPL)?

A/Luteal phase deficiency
B/Listeria monocytogenes exposure
C/Uterine anomalies
D/Parental chromosomal abnormalities✅
E/Environmental toxins

65.A hirsute, morbidly obese, 44-year-old woman presents with irregular periods. Pelvic sonogram demonstrates bilateral ovaries with multiple peripheral follicles. Which Of the following is the MOST appropriate management?
A/Laparoscopic bilateral ovarian cystectomy
B/Laparoscopic bilateral oophorectomy
C/ Oral contraceptives✅
D/ Danazol/gestrinone
E/Endometrial ablation

66.Which of the following is NoT seen in Turner syndrome?
A/Webbed neck
B/Pectus excavatum✅
C/Shield chest
D/Short stature
E/Coarctation of the aorta

67.A 6-year-old girl is brought to the office by her mother with signs of early sexual development. What is the MOST likely diagnosis of her precocious puberty?
A/McCune-Albright syndrome
B/Polycystic ovarian syndrome (PCOS),
C/Hormonally-active ovarian tumor
D/ Central nervous system (CNS) tumor
E/ldiopathic✅

68.An 88-year-old, medically infirm patient is brought by her daughter from a nursing home with worsening pelvic pain. Examination reveals uterine procidentia. What is the MOST appropriate intervention?
A/Colpoclesis✅
B/Abdominal sacrocolpopexy

C/ Sacrospinous ligament suspension
D/Total vaginal hysterectomy
E/Ring pessary

69.Which of the following is NOT evaluated during simple cystometry?
A/ Stress incontinence
B/ Detrusor overactivity
C/ Measurement of first sensation to void
D/ Bladder capacity
E/ Intra-abdominal pressure✅

70.A 79-year-old woman is diagnosed with urge incontinence. Which of the following NOT an appropriate treatment option?
A/Tension-free transvaginal tape (TVT) sling✅
B/Behavioral modification
C/Oxybutynin chloride
D/Detrusor injection with botulinum toxin
E/All of the above are appropriate

71.Which type of incision involves detaching the rectus tendon from the pubic symphysis?
A/Chemney✅
B/ Maylard
C/Pfannenstiel
D/ "j"-incision
E/All of the above

72.Which of the following is the MOST accurate statement about spontaneous miscarriage?
A/Most frequently occurs in second trimester
B/Chromosomal anomalies are rare
C/Happens to more than half of all conceptions✅
D/ Young maternal age is a risk factor
E/Triples the likelihood of miscarriage at next pregnancy

73..25-year-old woman presents with a 2.5-cm, firm, mobile breast mass. What is the MOST likely diagnosis?

A/Fibrocystic breast disease
B/Fibroadenoma✅
C/Invasive ductal breast carcinoma
D/Mastodynia
E/Duct ectasia

74.Which of the following is MOST accurate about vasectomy?
A/ Immediately effective
B/Requires general anesthesia
C/Easily reversible
D/ Requires postoperative semen analysis✅
E/Surgical interruption of the urethra

75.Which method is the MosT reliable for prevention of pregnancy?
A/Rhythm method
B/Coitus interruptus
C/ Postcoital douching
D/Prolonged breastfeeding
E/None of the above✅

76.morbidly obese, 32-year-old woman presents for contraceptive counseling. Which of the following is NOT absolute contraindication to use of oral contraceptives (OCs)?
A/ History of pulmonary embolism
B/ Hepatitis C carrier
C/ One pack per day smoker✅
D/ Estrogen-receptor-positive breast cancer
K/ Irregular uterine bleeding

77.22-year-old 6o with biopsy-confirmed endometriosis continues to have disabling dysmenorrhea for 3 days at the time of menses despite cyclic oral contraceptives (OCs). What is the BEST management option?
A/Endometrial ablation
B/Uterine artery embolization
C/Danazol/gestrinone
D/ GnRH agonist (Lupron)
E/Continuous OCs✅

78.Which of the following is NOT a theory to explain the pathogenesis of endometriosis?
A/ Retrograde menstruation
B/Celomic metaplasia
C/Hematogenous dissemination
D/Lymphatic exfoliation✅
E/Autoimmune disease

79.Which of the following non-malignant adnexal masses can give rise to Meigs syndrome?
A/ Ovarian fibroma✅
B/Paratubal cyst
C/Hydrosalpinx
D/ Endometrioma
E/Mature cystic teratoma

80.Which of the following non-malignant adnexal masses is derived from all three germ cell layers?
A/Ovarian fibroma
B/ Paratubal cyst
C/ Hydrosalpinx
D/ Endometrioma
E/ Mature cystic teratoma✅

81.Which of the following is NOT a structural anomaly seen in diethylstilbestro (DES)-exposed women?
A/ Cervical cockscomb
B/Cervical ectropion
C/Cervical hypoplasia
D/Vaginal adenosis
E/ Vaginal stenosis✅

82.A 66-year-old woman presents with vulvar pruritis and examination shows a parchment-like whitish appearance. Biopsy is MOST likely to confirm what diagnosis?
A/ Vulvar vestibulitis

B/Lichen s sclerosis✅
C/ Lichen planus
D/Psoriasis
E/ldiopathic vulvodynia

83.23-year-old woman with confirmed pelvic inflammatory disease (PID) presents with right upper quadrant pain. What is the MOST likely etiology?
A/ Cholecystitis
Q/ Nephrolithiasis
C/ Perihepatic abscess
D/ Asherman syndrome
E/Fitz-Hugh-Curtis syndrome✅

84.of the following is NOT a risk factor for pelvic inflammatory disease (PID)?
A/Use of a diaphragm for contraception✅
B/ Frequent vaginal douching
C/ Multiple partners
D/ Recent IUD/IUS placement
E/ Prior episode of PID

85. 16-year-old girl presents with a fever (39°C), nausea, pelvic pain, and generalized malaise a few days after unprotected intercourse. What is the MOST likely diagnosis?
A/ Trichomoniasis
B/Primary herpes simplex infection✅
C/Syphilis
D/Bartholin gland abscess
E/Lymphogranuloma venereum

86.patient presents with vulvar irritation and pruritis. Speculum examination reveals a hyperemic, edematous, vaginal vault with odorless discharge. The pH is 4.0. What is the MOST likely diagnosis?
A/Bacterial vaginosis (BV)
B/Candidiasis✅
C/Trichomoniasis

D/Gonococcal vaginitis
E/Hydradenitis suppurativa

87.A 48-year-old woman presents to the l office with a several year history of low pelvic pain. Which of the following is the LEAST likely cause?
A/ Interstitial cystitis
B/ Adenomyosis
C/Ectopic pregnancy✅
D/ Herniated disc
E /Inflammatory bowel

88.An 18-year-old G2P1 received l intramuscular (IM) methotrexate on Monday for a confirmed ectopic pregnancy with BhCG of 3,500 mlU/mL.She returns on Thursday with additional pelvic discomfort and a fhCG of 4,100 mlU/mL. Which of the following management options is most appropriate?
S/Second dose of methotrexate
B/ Transvaginal sonogram
C/ Laparoscopic salpingostomy
D/Repeat BhCG in 48 hours
E /Repeat BhCG in 72 hours✅

89.A 24-year-old G1Po presents to the office at 6 weeks gestation with a ß human chorionic gonadotropin (BhC6) 16,000 mlu/mL and mild left lower quadrant pain. Transvaginal sonogram confirms the presence of an unruptured 3-cm ectopic pregnancy without fetal cardiac motion. Which of the following is the best management option?
A/Methotrexate 30 mg/m2 intramuscularly
B/Methotrexate 50 mg/m2 intramuscularly
C/Culdocentesis
D/ Laparoscopic salpingostomy✅
K/Repeat BhCG in 48 hours

90.Which of the following is NOT a risk factor for ectopic pregnancy?
A/Prior history of pelvic inflammatory disease (PID)
B/Prior history of infertility
C/Prior history of uterine fibroids✅

D/ Prior history of ectopic pregnancy
E/Prior history of tubal surgery

91.A 46-year-old, non-pregnant, morbidly obese woman presents with irregular periods over the past 6 months. Which of the following represents the most appropriate initial management in this setting?
A/Oral contraceptives
B/Endometrial ablation
C/Office hysteroscopy
D/ Dilation and curettage (D&C)
E/Endometrial biopsy✅

92.Which structure produces gonadotropins?
A/Hypothalamus
B/Anterior pituitary✅
C/Posterior pituitary
D/ovaries
E/Endometium

93.Premenstrual syndrome (PMS) has a constellation of possible symptoms. Which of the following would NOT be expected?
A/ Abdominal bloating
B/ Weight loss✅
C/ Constipation
D/ Anxiety
E/ Breast tenderness

94.Which structure provides the major support to the uterus and cervix?
A/Round ligaments
B/Broad ligaments
C/Cardinal ligaments✅
D/ Uterosacral ligaments
E/ Scarpa fasdia